

RESUSCITATION SYSTEM OF CARE

Comprehensive Project Management Plan & Implementation Index

A Generalized Framework for Fire & EMS Agencies

Aligned with Resuscitation Academy Project Management Principles



Prepared for: Fire & EMS Agencies — United States
Document Type: Project Management Index — Cardiac Arrest Survival Initiatives
July 2026

Included Project Areas:

1. Public Access Defibrillation (PAD) Programs
2. Public Access Defibrillation Embedded Within Police
3. Telephone CPR (T-CPR) Quality Improvement
4. High-Performance CPR (HPCPR) Program for EMS Agencies
5. Quality Improvement Program: Dispatch/T-CPR and EMS Agencies
6. Quality Improvement / Quality Assurance for Cardiac Arrest Cases
7. Multi-Agency HPCPR Work Groups
8. Cardiac Arrest Registry to Enhance Survival (CARES) Program
9. Community CPR Programs

How to Use This Document

This plan is organized into eight project areas, each modeled on the core components of the Resuscitation Academy's approach to improving out-of-hospital cardiac arrest (OHCA) survival. Every project area follows the same consistent structure so that any Fire/EMS agency, regardless of size, budget, or geographic setting, can locate and apply the relevant guidance. This document is intended to provide a general framework and should not be read as prescriptive or rigid. Rather, it serves as a comprehensive reference guide. The mentor's role is to help each team identify the components specific to its own project.

Each project section includes:

- Overview & Purpose: the clinical/operational rationale and scope of the initiative.
- Cost Considerations & Budgeting: capital, recurring, staffing, and funding-source guidance.
- Equipment & Procurement: devices, software, and systems required.
- Agency Cooperation & Interagency Coordination: cross-agency agreements and data sharing.
- Administrator & Command Staff Buy-In: how to secure leadership and policy support.
- Stakeholder & Community Buy-In: how to engage the public, labor groups, and partner organizations.
- Implementation Milestones: a minimum of five sequential milestones required for successful implementation, presented in table form with success criteria.

Content is intentionally generalized so that it can be tailored to any jurisdiction's size, governance structure, budget, and existing infrastructure. Dollar figures, staffing levels, and timelines are illustrative starting points and should be validated against local conditions, state EMS regulations, and current vendor pricing.

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1. Public Access Defibrillation (PAD) Programs

Placing AEDs in high-risk, high-traffic public locations and training laypersons to use them before EMS arrival.

1. Overview & Purpose

Public Access Defibrillation (PAD) programs place automated external defibrillators (AEDs) in locations where out-of-hospital cardiac arrest (OHCA) is likely to occur: transit hubs, shopping centers, schools, government buildings, gyms, places of worship, and large-venue assembly spaces, and train laypersons/first responders to use them in the critical minutes before EMS arrival.

Every minute without defibrillation after collapse reduces survival probability by approximately 7–10%. A well-designed PAD program closes the gap between collapse and first shock, working in tandem with early recognition, 9-1-1 activation, and high-quality bystander CPR.

This is intended as a generalized framework that any Fire/EMS agency, regardless of size or resources, can adapt to its jurisdiction.

[Resuscitation Academy Toolkit Community PAD](#)

2. Cost Considerations & Budgeting

- **AED Unit Cost:** Budget \$1,200–\$2,500 per AED unit depending on model, plus cabinet/wall-mount enclosures (\$150–\$600), signage, and alarmed cases for outdoor or unsecured locations.
- **Consumables & Lifecycle Costs:** Recurring costs include replacement electrode pads (2–5 year shelf life), batteries, and unit replacement at end-of-service-life (typically 8–10 years); build a rolling replacement reserve into the annual budget.
- **Registry & Software Fees:** Consider account for subscription costs for AED location-registry/management platforms that track device status, maintenance, and integration with 9-1-1 dispatch mapping if applicable.
- **Training & Materials:** Include per-session costs for CPR/AED course materials, manikins, and instructor time if training is delivered in-house rather than contracted.
- **Grant & Funding Sources:** Identify FEMA Assistance to Firefighters Grants (AFG), state EMS-for-Children or Heart Safe Community grants, corporate/foundation sponsorships, and local service-club donations to offset capital costs.

Commented [ML1]: Registry and Software fees are optional- smaller agencies might choose to do themselves. Integration with 911 dispatch mapping would require a fee.

3. Equipment & Procurement

- **Site Selection Criteria:** Prioritize locations using historical cardiac-arrest incidence data, occupancy/foot-traffic volume, and distance-to-AED response time modeling (target under 3 minutes retrieval time).
- **Device Standardization:** Select a single AED make/model (or limited approved list) across the jurisdiction to simplify training, maintenance contracts, and parts inventory.
- **Maintenance Program:** Establish a documented preventive-maintenance schedule (self-test logs, pad/battery expiration tracking) with a single responsible point of contact per site.
- **Integration with Dispatch/CAD:** Register every device location with the 9-1-1 center's computer-aided dispatch (CAD) system and/or a smartphone AED-locator app so dispatchers can direct callers to the nearest unit.

4. Agency Cooperation & Interagency Coordination

- **Facility Partnerships:** Formalize agreements (MOUs) with schools, malls, transit authorities, airports, and large employers to host, maintain, and self-report on devices.

- **Law Enforcement & Security Coordination:** Coordinate with facility security teams and law enforcement so responding officers know device locations and are trained to retrieve/apply AEDs.
- **Regional Data Sharing:** Share device-location data with neighboring EMS agencies and the regional 9-1-1 authority to support mutual aid and cross-jurisdictional response.

5. Administrator & Command Staff Buy-In

- **Executive Sponsorship:** Secure written endorsement from the Fire Chief/EMS Medical Director framing PAD expansion as a core survival initiative, not a discretionary add-on.
- **Budget Advocacy:** Prepare a cost-benefit brief (lives saved per dollar spent, liability protection under Good Samaritan/AED statutes) for city council, county commission, or agency leadership.
- **Policy Adoption:** Obtain administrative approval of a formal PAD policy defining device standards, maintenance responsibility, and data-reporting requirements.

6. Stakeholder & Community Buy-In

- **Public Education Campaigns:** Partner with local media, schools, and civic organizations to promote AED awareness ('Take 10 to Save a Life'-style campaigns) and normalize bystander action.
- **Business & Property Owner Engagement:** Present cost, liability protection, and reputational benefits to property managers and business owners to encourage voluntary AED acquisition.
- **Survivor & Community Advocates:** Feature cardiac-arrest survivors and family testimonials at public events and council meetings to build sustained community support and funding momentum.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Community Risk & Gap Assessment	Complete a jurisdiction-wide analysis of OHCA incidence, existing AED locations, and coverage gaps; identify priority placement sites.
2	Funding & Procurement Secured	Secure capital funding (grant, budget line, or donation) and execute procurement of standardized AED units, cabinets, and signage for priority sites.
3	Site Agreements & Installation	Execute host-site MOUs, install devices, register each unit in the AED locator/registry and CAD system, and post public signage.
4	Training Rollout	Deliver CPR/AED training to facility staff, security personnel, and community members at and near each installation site; track completion rates.
5	Maintenance & Compliance Program Live	Launch a recurring inspection/maintenance cycle with assigned site champions and documented self-test/pad-expiration tracking.
6	Outcome Tracking & Program Review	Establish a process to capture AED-use events, link to patient outcomes (via CARES or equivalent registry), and conduct an annual program review with expansion recommendations.

Commented [ML2]: Would you want to add anything about advocating for AED placement laws, if they don't exist?

Commented [PH3R2]: @Tegan Hampton, your thoughts here? I think we can mention it but it feels like advocating for AED placement laws would be another project type?

Commented [TH4R2]: @Peder Humlen I agree with you. We can mention it, but the scope of an advocacy project would be much larger, and definitely a project of its own.

2. Public Access Defibrillation Embedded Within Police

Equipping and training lawenforcement patrol officers as first-responder AED/CPR providers to reduce time-to-first-shock.

1. Overview & Purpose

In many jurisdictions, patrol officers are dispersed throughout the community and frequently arrive at a cardiac-arrest scene before EMS. Embedding AEDs and high-performance CPR training into police operations creates an additional tier of first response that meaningfully reduces collapse-to-shock intervals.

This initiative requires close cross-agency planning between the Fire/EMS agency (as the medical authority and training resource) and the law-enforcement agency (as the operational host), and is generalized here for adoption across any city, county, or state police/sheriff partner.

[Resuscitation Academy Toolkit Police Defibrillation](#)

2. Cost Considerations & Budgeting

- **In-Vehicle AED Units:** Budget for one AED per patrol vehicle (or per officer, if applicable), typically \$1,200–\$2,500 each, plus vehicle-mount brackets rated for temperature extremes and vibration.
- **Fleet-Wide Scale Costs:** Model total cost against fleet size; phase deployment (e.g., highest call-volume beats/precincts first) if full fleet equipping exceeds available budget in year one.
- **Consumables & Replacement Reserve:** Include recurring pad/battery replacement and 8–10 year device replacement cycles in the police department's or EMS agency's capital plan.
- **Training Cost-Sharing:** Negotiate whether the Fire/EMS agency or police department funds instructor time, course materials, and CPR/AED card renewals; many jurisdictions cost-share via an interagency agreement.
- **Grant Opportunities:** Pursue Bureau of Justice Assistance (BJA) grants, state officer-safety grants, and AFG funding, many of which explicitly support officer-carried AEDs.

3. Equipment & Procurement

- **Standardized Device Selection:** Align the police AED model with the Fire/EMS agency's standard device where feasible to streamline joint training and maintenance.
- **Vehicle Integration:** Specify mounting location and access procedure so devices are retrievable within seconds under duty-belt and vehicle-console constraints.
- **Maintenance Ownership:** Assign a police-department equipment officer as the maintenance point of contact, with EMS providing technical oversight and periodic device audits.

4. Agency Cooperation & Interagency Coordination

- **Interagency Agreement (MOU):** Formalize roles, medical oversight, training responsibilities, equipment ownership, and data-sharing terms between the Fire/EMS agency and the police department.
- **Medical Direction:** Ensure the EMS Medical Director provides protocol oversight and periodic skills verification for officer AED/CPR use, consistent with state EMS scope-of-practice rules.
- **Joint Dispatch Protocols:** Update CAD/dispatch protocols so police units are dispatched concurrently with EMS on suspected cardiac-arrest calls ('closest unit' response).

5. Administrator & Command Staff Buy-In

- **Chief-to-Chief Alignment:** Secure joint endorsement from the Fire Chief and Police Chief/Sheriff, framing the program as a shared public-safety mission rather than an unfunded mandate on either department.

- **Union & Labor Engagement:** Brief police labor representatives early on training-time expectations, liability protections, and equipment responsibilities to prevent implementation friction.
- **Performance Metrics for Leadership:** Provide command staff with clear metrics (officer-initiated shocks, time-to-first-shock reduction) to sustain political and budgetary support.

6. Stakeholder & Community Buy-In

- **City/County Governing Body:** Present the program to city council or county commission as a cross-departmental public-safety investment with measurable survival impact.
- **Community & Advocacy Groups:** Engage heart-health advocacy organizations and survivor networks to publicly support officer-AED funding requests.
- **Media & Public Awareness:** Publicize officer-saves through coordinated Fire/Police/EMS press releases to reinforce community confidence and sustain funding.
- **Recognition & Feedback:** Provide mechanisms to provide officer/deputy recognition and feedback to Police Chiefs/Sheriffs and staff peers.

Commented [ML5]: Additionally, provide officer recognition/feedback to Police Chief and staff peers.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Interagency MOU Executed	Fire/EMS and police leadership finalize and sign an MOU defining medical oversight, funding split, training responsibility, and data sharing.
2	Equipment Procurement & Vehicle Installation	Procure standardized AEDs and install vehicle mounts; prioritize highest call-volume patrol areas for phase one.
3	Officer Training Completed	Train all participating officers in high-performance CPR and AED use, with EMS providing initial and refresher instruction.
4	Dispatch Protocol Update	CAD/dispatch center updates protocols to co-dispatch police with EMS on cardiac-arrest calls and confirm radio/handoff procedures.
5	Pilot Response Review	Conduct a 90-day pilot in initial deployment areas; review response times, officer AED use, and any equipment/process issues before fleet-wide rollout.
6	Full Deployment & Sustainment	Expand to full fleet coverage, integrate AED maintenance into routine vehicle inspections, and establish annual skills re-certification.

3. Telecommunicator CPR (T-CPR) Quality Improvement

Ensuring 9-1-1 dispatchers rapidly recognize cardiac arrest and deliver high-quality, scripted CPR instructions to bystanders.

1. Overview & Purpose

Telecommunicator CPR (T-CPR), also called dispatcher-assisted CPR (DA-CPR), is the process by which 9-1-1 telecommunicators recognize possible cardiac arrest, and deliver CPR pre-arrival instructions before first responder arrival.

T-CPR quality improvement focuses on rapid recognition of cardiac arrest, coaching high-quality CPR (e.g., compression rate, minimizing interruptions), overcoming barriers that may delay or prevent the delivery of care to a patient, and reducing preventable delays. Each is independently linked to bystander CPR rates and survival. This section is written for adoption by any Public Safety Answering Point (PSAP)/9-1-1 emergency communications center regardless of call volume.

[Resuscitation Academy Toolkit T-CPR Quality Improvement](#)

[Resuscitation Academy Toolkit T-CPR Program](#)

[Resuscitation Academy High Performance TCPR Resource Guide](#)

[Resuscitation Academy Best Practices](#)

2. Cost Considerations & Budgeting

- **CAD/QA Software:** Budget for computer-aided dispatch (CAD) protocol modules (e.g., structured cardiac-arrest triage scripts) and call-audit/QA software capable of time-stamping key intervals (call receipt, arrest recognition, first compression instruction).
- **Recording & Review Systems:** Ensure phone recording systems support easy retrieval and clipping of calls for case review and dispatcher coaching; budget for any needed system upgrades.
- **Telecommunicator Training Time:** Account for paid training/overtime hours for initial T-CPR protocol training and ongoing simulation-based refreshers.
- **Staffing for QI Function:** Budget for a dedicated (even if part-time) T-CPR QI coordinator or AI-assisted case review solution to review calls, track metrics, monitor protocol compliance, identify opportunities for improvement, and support human-led feedback and coaching.
- **Simulation & Skills Assessment:** Budget for recurring T-CPR simulation, skills assessment, and debriefing to reinforce best practices, identify performance gaps, and maintain competency.

3. Equipment & Procurement

- **Standardized Cardiac Arrest Protocol:** Adopt a structured, evidence based emergency medical dispatch protocol that facilitates rapid recognition of cardiac arrest and provides scripted telecommunicator CPR (T-CPR) instructions.
- **Call Review & Metrics Dashboard:** Implement a system to flag and review all suspected cardiac-arrest calls, tracking recognition time, time-to-first-compression, coaching high-quality CPR (e.g., compression rate, interruptions), delays and barriers, and preventable delays.
- **Simulation Training Tools:** Use recorded call scenarios and simulation manikins/software for scenario-based training and competency verification.

4. Agency Cooperation & Interagency Coordination

Commented [JB6]: Not always compression only

Commented [JB7]: This would be more inclusive as "first responder." Not all systems have EMS (some systems' EMS is privatized and handled by companies like AMR), and Fire is the primary public safety responder. And sometimes law is first on scene. First responder, or something other than just EMS is more inclusive.

Commented [PH8R7]: AMR is still considered EMS as is fire-department bls only. EMS is a generic term in this case

Commented [JB9]: I think it's important that cost does not become a barrier to improvement. To conduct QI all you really need is a pencil, paper, and a way to keep track of time. Things like software and even CAD in some systems may not be something they have or will have. But, they can still conduct these reviews.

Commented [JB10]: One thing I worry about here that for our smaller communities, or even those who come to the RA without their ECC leadership, everything I see here is \$\$\$\$, and that is something (especially the comm centers do not have). Is there a "good, better, best" model that could potentially be followed? Again, all you need is a pencil, paper, and a way to keep time and you can do QI. I just worry this is overly complex and expensive and could be a barrier for communities

Commented [PH11R10]: This is only for considerations, not all inclusive. All systems are different and may have to take into consideration of these specific budgetary concerns. I can definitely add a basic model, but even that comes with \$\$\$\$ of personnel costs and appropriating resources to do it

Commented [MC12]: There are numerous companies currently offering automated case review via AI.

Commented [JB13]: I think this will also discourage or prevent participation for some. This can be done by staff on duty, such as supervisors or CTOs, or even officers on light duty.

Commented [MC14R13]: agree with both of your points on discouraging/preventing due to budget. I added sims, which can occur in the same manner. We should have a header/disclaimer that states that a lot of this can be done with very minimal costs for staffing when necessary.

Commented [PH15R13]: Absolutely, but we would be remiss to ignore budgetary costs as a potential barrier. This is nothing more than a "consideration" component. As a Chief, cost is ALWAYS a question I have to ask, "what i ... [1]

Commented [JB16]: Criteria-based dispatch is a product name.

Commented [PH17R16]: wow, really?

Commented [PH18R16]: looking online and it's not a company that I can see, it's a dispatch strategy that is common like EMD.

Commented [JB19]: See list above

- **Dispatch-EMS Feedback Loop:** Establish a routine process for EMS field crews and the medical director to provide EMS-confirmed cardiac arrest data and patient outcome information to the dispatch center for case review and quality improvement.
- **PSAP-Fire/EMS Data Sharing:** Where dispatch is operated by a separate agency (regional 9-1-1 authority, police-run PSAP), formalize a data-sharing agreement to permit joint QI review of cardiac-arrest calls.
- **Regional Protocol Alignment:** Coordinate with neighboring PSAPs, EMS agencies, and the state EMS office to promote consistent T-CPR best practices, performance benchmarks, and quality improvement initiatives across the region.

5. Administrator & Command Staff Buy-In

- **PSAP Director & EMS Medical Director Alignment:** Secure joint sign-off from the PSAP director and EMS medical director on the standardized protocol and QI review process.
- **Performance Reporting to Leadership:** Provide monthly/quarterly dashboards to agency leadership showing T-CPR compliance rates and their correlation with bystander CPR rates and survival.
- **Policy & Staffing Support:** Obtain administrative approval for protected QI review time and any additional staffing needed to sustain call-review workload.

6. Stakeholder & Community Buy-In

- **Recognition & Culture:** Publicly recognize 'saves' and high-performing shifts to build morale and reinforce the value of T-CPR among telecommunicator staff.
- **EMS Field Provider Engagement:** Involve field paramedics/EMTs in reviewing dispatch-to-scene handoffs so they understand and support the T-CPR process.
- **Community & Governing Board Awareness:** Report T-CPR performance improvements to the governing board/council alongside PAD and CARES outcomes to demonstrate system-wide cardiac-arrest survival gains.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Protocol Adoption	Select and formally adopt a standardized, scripted T-CPR dispatch protocol.
2	Baseline Data Collection	Establish call-recording review capability and collect baseline T-CPR quality metrics and key performance indicators, including recognition time and time to first compression.
3	Dispatcher Training Completed	Train 100% of telecommunicators on the protocol and reinforce patient care through simulation-based cardiac arrest scenarios.
4	QI Review Process Launched	Implement a recurring (e.g., monthly) case-review process with individualized telecommunicator feedback and trend reporting to supervisors and quality improvement leadership.
5	Performance Benchmarks Achieved	Meet or exceed target benchmarks established by agency/region (e.g., cardiac-arrest recognition within 90 seconds, first-compression instruction within 150 seconds) on a sustained basis.

Commented [MC20]: In this highlighted area, there is reference to "PSAP", "Dispatch", "9-1-1 authority", and "9-1-1 center". For consistency, I recommend that only one is chosen and used throughout the document.

Commented [PH21R20]: good catch, I'll update. PSAP is generic enough. Thank you

Commented [MC22]: Consider broadening this to include T-CPR quality metrics and key performance indicators rather than only compliance rates. Also consider whether these dashboards should be provided to both agency leadership, EMS agencies, and the EMS medical director; and clarify who is responsible for developing them and incorporating EMS-derived outcome data.

Commented [PH23R22]: I think that is covered in another section in the quality improvement document, I'll verify

Commented [MC24]: The benchmark examples shown here reflect high-performance system goals rather than minimum acceptable performance. The current American Heart Association T-CPR performance recommendations identify minimum acceptable KPI goals of approximately 90 seconds for cardiac arrest recognition and 150 seconds to first compression, while 60 seconds and 90 seconds, respectively, represent high-performance system benchmarks. Achieving a median 90-second time-to-first-compression across all cardiac arrest calls is exceptionally difficult. Consider using more general language (e.g., "meet or exceed established T-CPR performance benchmarks") or clarifying that these example times represent high-performance system goals.

Commented [PH25R24]: I'll rephrase to say agency target benchmarks and keep the meet/exceed. That's why there is an e.g to show an example of, but I'll make that more clear. Thanks Mark

#	Milestone	Description / Success Criteria
6	Outcome Integration	Link T-CPR case data with patient outcome/registry data (e.g., CARES) to demonstrate survival impact and guide continued protocol refinement.

4. High-Performance CPR (HP-CPR) Program for EMS Agencies

Building disciplined, metrics-driven team-based CPR delivery among EMS providers to maximize compression quality and minimize interruptions.

1. Overview & Purpose

High-Performance CPR (HP-CPR) is a “pit-crew” styled, BLS focused, choreographed approach to resuscitation with defined roles (Compressor, Airway, AED/Monitor, Time keeper). Pre-briefed rotations minimize compression interruptions, maximize compression fraction, and ensure consistent depth/rate/recoil.

This initiative equips EMS agencies of any size to train, measure, and continuously improve team-based resuscitation performance using objective feedback data rather than subjective impression.

2. Cost Considerations & Budgeting

- **Consider agency need for CPR feedback/monitoring devices, if necessary, consider budget implications**
- **Training Manikins:** Include high-fidelity feedback-capable CPR manikins with pad-based feedback for initial and recurring team-based skills training. See Equipment for HP-CPR training on RA website.
- Real-time CPR feedback/monitoring devices or defibrillator-integrated feedback modules may be helpful to provide rate, depth, recoil, and compression fraction feedback.
- **Simulation Time & Overtime:** Budget paid training hours for scenario-based pit-crew drills, ideally recurring monthly or quarterly rather than an annual one-time session.
- **Data Review Software:** Account for software/licensing to download, aggregate, and analyze compression-fraction and CPR-quality data from cardiac-arrests.

3. Equipment & Procurement

- High Quality Feedback Manikins: as described above
- **Defibrillator/Monitors:** Devices capable of post-event data download are essential for every response unit. An AED or monitor-defibrillator platform capable of real-time CPR feedback may be helpful.
- **Role-Specific Kits/Checklists:** Develop pre-positioned, role-labeled equipment (airway kit, IV/IO kit, medication box) so each crew member's station is immediately identifiable on arrival.
- **Timer/Metronome Tools:** Provide audible/visual compression-rate metronomes (device-integrated or standalone) to reinforce consistent rate during training and live calls.

4. Agency Cooperation & Interagency Coordination

- **Multi-Unit Response Coordination:** Establish clear roles and communication protocols for scenarios involving multiple responding units (first-due engine/ambulance plus mutual aid) to avoid confusion, role duplication, or gaps.
- **Fire-Based First Responder Integration:** Where fire crews respond alongside ambulances, pre-define which crew assumes which pit-crew role on arrival.
- **Medical Director Protocol Alignment:** Ensure HPCPR choreography is embedded in written clinical protocols approved by the EMS medical director.

5. Administrator & Command Staff Buy-In

- **Data-Driven Leadership Case:** Present baseline compression-fraction and CPR-quality data to command staff to justify investment in feedback devices and recurring training time.

- **Training Time Allocation:** Obtain administrative commitment to protected, recurring (not one-time) training time as HPCPR skills decay without regular drilling.
- **Performance Accountability Structure:** Establish administrative support for non punitive individual and crew-level performance review using objective CPR-quality metrics.

6. Stakeholder & Community Buy-In

- **Field Provider Ownership:** Involve field paramedics/EMTs and field training officers in designing role assignments and drill scenarios so the program reflects real operational constraints.
- **Union/Labor Communication:** Communicate early with labor representatives about training-time expectations and how performance data will be used.
- **Hospital Partner Engagement:** Share aggregate HPCPR performance and outcome trends with receiving hospital resuscitation teams to reinforce the pre-hospital-to-hospital continuum of care.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Baseline Performance Assessment	Deploy CPR feedback devices agency-wide and collect baseline compression-fraction, rate, and depth data across real cardiac-arrest responses.
2	HP-CPR Model Adopted	Formally adopt a role-based resuscitation protocol, approved by the EMS medical director, defining roles, rotations, and communication cues.
3	Initial Team Training Completed	All response crews complete HP-CPR training using role based model and scenario-based drills with feedback manikins.
4	Recurring Drill Schedule Established	Implement a sustained (monthly/quarterly) training cadence with documented attendance and skills verification.
5	Performance Benchmarks Met	Achieve target compression-fraction benchmarks (e.g., $\geq 80\%$ chest-compression fraction) on a sustained, agency-wide basis.
6	Continuous QI Feedback Loop	Establish individualized post-call debriefs and periodic aggregate performance reporting to crews and administration to drive ongoing improvement.

5. Quality Improvement (QI) Program — Dispatch/T-CPR and EMS Agencies

A dual-track QI structure: one track for 9-1-1 dispatch/T-CPR performance, and a second track for EMS agency clinical and operational performance.

1. Overview & Purpose

Sustainable improvement in cardiac-arrest survival requires ongoing, structured Quality Improvement (QI) rather than a one-time protocol rollout. This project establishes two parallel, complementary QI tracks: (A) a Dispatch/T-CPR QI Track focused on 9-1-1 call-handling performance, and (B) an EMS Agency QI Track focused on field clinical and operational performance. Both tracks feed into a shared cardiac-arrest performance-improvement governance structure, with each sub-project detailing its own cost, equipment, agency-cooperation, administrator buy-in, and stakeholder buy-in considerations below.

[Resuscitation Academy Toolkit T-CPR Quality Improvement](#)

Sub-Project 5A: Dispatch & Telephone-CPR Quality Improvement Track

- **Focus:** 9-1-1/PSAP recognition speed, T-CPR protocol compliance, and time-to-first-compression instruction (instructions does not = first compression) (builds directly on the T-CPR program in Section 3, but formalizes it as an ongoing QI cycle rather than a one-time initiative).
- **Core Activities:** Monthly case sampling and audit of suspected cardiac-arrest calls. The long-term goal is review of 100% of EMS-confirmed OHCA cases. However, that may be too much for agencies just beginning a T-CPR quality improvement program, and representative case sampling may be a more practical starting point. Include dispatcher-level scorecards; supervisor-led coaching sessions; trend analysis by shift and by call type.
- **Cost Considerations:** Dedicated QI coordinator time or an AI-assisted case review solution, call-audit/QA software capable of tracking T-CPR performance metrics, and protected supervisor and QI staff time for individualized coaching, and dissemination of quality improvement findings through agency-wide training and education
- **Equipment/Systems:** CAD reporting modules capable of exporting timestamped call data; secure case-review recording access; and a system for case review, performance tracking, and reporting (ranging from spreadsheets to dedicated quality improvement software).
- **Agency Cooperation:** Data-sharing agreement between the PSAP/dispatch authority and the EMS agency; shared access to EMS-confirmed cardiac arrest and patient outcome data to close the feedback loop.
- **Administrator Buy-In:** PSAP director and EMS medical director jointly approve review criteria and protected QI time.
- **Stakeholder Buy-In:** telecommunicator union/labor engagement; recognition programs for high performers to build a positive QI culture.

Commented [MC26]: Due to my recommended changes in the previous section, consider updating this focus statement to align with the revised terminology (e.g., cardiac arrest recognition, T-CPR quality metrics, key performance indicators, and time-to-first-compression).

Commented [PH27R26]: Copy

Sub-Project 5B: EMS Agency Clinical & Operational Quality Improvement Track

- **Focus:** field clinical performance (protocol adherence, HP-CPR metrics, medication administration, airway management) and operational performance (response times, unit availability, documentation completeness).
- **Core Activities:** case-based chart review (ePCR audits), CPR-feedback-device data review, crew debriefs after significant cardiac-arrest calls, and quarterly performance-trend reporting to command staff.
- **Cost Considerations:** ePCR/QI software licensing, dedicated QI officer/coordinator staffing, and paid time for peer-review committee participation.

Commented [MC28]: Consider broadening this recommendation beyond review criteria. Administrator buy-in should include support for the overall T-CPR quality improvement program, including dedicated staff time and resources needed to sustain ongoing case review, coaching, education, and performance improvement.

Commented [PH29R28]: Agreed, I'll add a component that says such as: and list those out as examples.

Commented [MC30]: I like the first part of this recommendation. The second part could better reflect the broader goal of fostering a "non-punitive" quality improvement culture. Recognition programs are an important component, but they should be presented as one element of a culture that values learning, coaching, continuous improvement, and recognition.

- Equipment/Systems: ePCR platform with QI-flagging capability; CPR feedback-device data export integration; secure peer-review documentation storage.
- Agency Cooperation: coordination with receiving hospitals for outcome data (survival to discharge, neurological status) and with the regional EMS authority for benchmark comparison.
- Administrator Buy-In: command staff commitment to a non-punitive peer-review process protected under applicable state peer-review confidentiality statutes.
- Stakeholder Buy-In: field provider involvement in designing review criteria; sharing de-identified trend data agency-wide to demonstrate value and build trust in the process.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	QI Governance Structure Established	Charter a joint QI committee with representation from dispatch, EMS operations, medical direction, and hospital partners; define roles and responsibilities, meeting cadence and reporting lines.
2	Data Systems Connected	Ensure CAD, ePCR, and (where available) hospital/registry data systems can be linked or cross-referenced for case-level QI review.
3	Case Review Process Implemented (Both Tracks)	Launch recurring case review for both dispatch/T-CPR calls and EMS clinical responses, with standardized review criteria and feedback documentation.
4	Benchmark Metrics Defined & Tracked	Adopt nationally recognized benchmarks (e.g., CARES and American Heart Association metrics) for both tracks and begin trend reporting.
5	Feedback Loops to Frontline Staff	Deliver individualized and aggregate performance feedback and educational updates to telecommunicators and field providers on a recurring, non-punitive basis.
6	Annual QI Report & Improvement Plan	Publish an annual joint QI report to agency leadership and governing bodies with identified gaps, accomplishments, and a documented improvement plan for the next cycle.

6. Quality Improvement / Quality Assurance (QI/QA) for Cardiac Arrest Cases

A structured case-review framework that audits every out-of-hospital cardiac arrest against evidence-based benchmarks, chain-of-survival intervals, and documentation completeness.

1. Overview & Purpose

While Section 5 establishes the ongoing QI governance tracks for dispatch and EMS operations broadly, this section defines the specific case-level Quality Assurance (QA) process applied to every individual out-of-hospital cardiac arrest (OHCA) event. The goal is 100% case capture and review, not sampling, so no cardiac-arrest case goes unexamined.

This process is the clinical audit backbone that feeds data into the CARES registry (Section 8) and informs the multi-agency work groups (Section 7).

2. Cost Considerations & Budgeting

- **Chart Abstraction Staffing:** Budget for dedicated (even part-time) QA/registry-abstraction staff time to review 100% of cardiac-arrest ePCRs and abstract data into a standardized case-review template.
- **Software & Data Systems:** Include costs for ePCR QA-flagging modules, secure case-review documentation storage, and any registry-abstraction software licensing.
- **Medical Director Review Time:** Compensate or protect time for the EMS medical director/physician reviewer to sign off on flagged cases requiring clinical review.

3. Equipment & Procurement

- **Standardized Case-Review Template:** Adopt a structured audit tool covering chain-of-survival intervals (collapse-to-CPR, collapse-to-shock, collapse-to-ALS), Utstein-style variables, and protocol adherence.
- **Data Capture Integration:** Ensure CPR-feedback-device data, AED data, and CAD timestamps can be merged into a single case file for each cardiac-arrest event.

4. Agency Cooperation & Interagency Coordination

- **Hospital Outcome Data Sharing:** Establish data-sharing agreements with receiving hospitals to obtain survival-to-discharge and neurological (CPC/mRS) outcome data for every case, closing the pre-hospital-to-hospital loop.
- **Peer-Review Legal Protections:** Confirm the QA process operates under applicable state peer-review/quality-assurance confidentiality statutes, and coordinate with legal counsel across participating agencies.
- **Regional Benchmark Comparison:** Coordinate with the regional EMS authority and/or state EMS office to compare case-review findings against regional and national benchmarks.

5. Administrator & Command Staff Buy-In

- **Medical Director Endorsement:** Ensure the EMS medical director champions the QA process as the clinical authority responsible for protocol compliance and case sign-off.
- **Non-Punitive Review Culture:** Secure administrative commitment that case review is used for systems and individual coaching improvement, not disciplinary action, to encourage honest documentation and engagement.
- **Resourcing Commitment:** Obtain leadership approval for the staffing/time investment required to review every case rather than a sample, recognizing this as a patient-safety, not optional, function.

6. Stakeholder & Community Buy-In

- **Field Provider Engagement:** Involve field providers in case debriefs so review findings translate into practical, respected feedback rather than top-down criticism.
- **Cross-Agency QA Committee:** Where multiple agencies serve a region, form a joint QA committee to review cases with cross-jurisdictional handoffs (e.g., first responder engine plus transporting ambulance from different agencies).
- **Governing Body Reporting:** Provide periodic, de-identified aggregate QA findings to city/county leadership and the public to demonstrate accountability and continuous improvement.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	100% Case Capture Process Defined	Establish a workflow ensuring every OHCA event is automatically flagged in the ePCR/CAD system for mandatory QA review.
2	Standardized Review Template Adopted	Adopt and train reviewers on a structured, Utstein-style case-review template covering clinical and operational variables.
3	Data Integration Achieved	Successfully merge CAD, ePCR, AED/CPR-feedback-device, and hospital outcome data into a unified case file for each event.
4	Review Cycle Operational	Launch a recurring review cycle (e.g., within 30 days of each event) with medical director sign-off on flagged cases.
5	Feedback to Crews & Dispatch	Deliver case-specific and aggregate feedback to involved dispatch and field personnel on a consistent, non-punitive basis.
6	Outcome-Linked Reporting	Produce quarterly reports linking case-review findings to patient outcomes and submit summary data to the CARES registry (Section 8).

7. Multi-Agency High-Performance CPR (HP-CPR) Work Groups

Regional collaboratives of fire, EMS, and dispatch agencies that standardize resuscitation practice and jointly drive regional survival-rate improvement.

1. Overview & Purpose

Cardiac-arrest survival is a regional, not single-agency, outcome; patients cross jurisdictional lines through mutual aid, automatic-aid, and transport to shared hospital systems. Multi-agency HP-CPR work groups bring together fire departments, EMS agencies, 9-1-1 centers, and hospital partners across a region to standardize resuscitation protocols, share data, and jointly benchmark performance.

This model is adaptable to any regional configuration, a single county with multiple fire districts, a multi-county EMS region, or a state-level collaborative, and is intended to complement (not replace) each agency's internal HP-CPR program (Section 4).

2. Cost Considerations & Budgeting

- **Shared Coordinator/Facilitator:** Budget for a shared regional coordinator role (often funded jointly or hosted by a regional EMS authority) to organize meetings, aggregate data, and drive follow-through on action items.
- **Joint Training Events:** Include costs for periodic joint multi-agency simulation/training events, venue costs, and shared instructor resources.
- **Shared Data Platform:** Budget for a shared regional data-aggregation platform or registry subscription (which may overlap with CARES participation costs in Section 8) that allows cross-agency benchmarking.
- **Cost-Sharing Formula:** Negotiate an equitable cost-sharing formula among participating agencies (e.g., per-capita, per-call-volume, or flat-fee membership) to fund shared infrastructure.

3. Equipment & Procurement

- **Common Training and Protocol Standardization:** Work toward shared or harmonized HP-CPR training, protocols, common CPR-feedback-device data format, and compatible equipment standards across participating agencies to enable direct performance comparison.
- **Shared Reporting Templates:** Develop standardized regional scorecards/dashboards so every agency reports the same core metrics in the same format.

4. Agency Cooperation & Interagency Coordination

- **Regional MOU/Charter:** Establish a formal charter or MOU defining work-group governance, meeting frequency, data-sharing terms, and decision-making processes (e.g., consensus vs. majority vote).
- **Cross-Agency Medical Direction:** Coordinate among participating medical directors to align clinical protocols and resolve variance in practice standards.
- **Neutral Convening Body:** Where feasible, use a regional EMS authority, council of governments, or state EMS office as a neutral convener to reduce inter-agency competition or turf concerns.

5. Administrator & Command Staff Buy-In

- **Multi-Chief Sponsorship:** Secure sponsorship and active participation commitment from fire/EMS chiefs and dispatch directors across all participating agencies, the work group's credibility depends on leadership-level engagement, not just line-staff attendance.
- **Elected Official Briefings:** Brief city/county elected officials on the regional collaborative's goals to build political support for cost-sharing and cross-agency data disclosure.

- **Transparent Performance Reporting:** Establish leadership agreement on how comparative agency performance data will be shared, de-identified/aggregate initially, with a pathway to more transparent agency-level reporting as trust builds.

6. Stakeholder & Community Buy-In

- **Hospital & Health-System Partners:** Engage regional hospital resuscitation and cardiology teams as work-group members to close the full chain-of-survival loop and share outcome data.
- **Public Reporting & Regional Survival Goals:** Publish regional survival-rate trends and improvement goals to build public accountability and community support across the region.
- **Academic/Research Partners:** Where available, involve university or research partners to support data analysis and publish findings that reinforce the work group's credibility and attract funding.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Work Group Chartered	Convene founding member agencies and execute a regional charter/MOU defining governance, data-sharing terms, and meeting cadence.
2	Baseline Regional Data Compiled	Aggregate baseline HP-CPR and outcome data from all participating agencies into a common reporting format.
3	Training and Protocol Harmonization Agreement	Reach consensus among medical directors on harmonized or mutually compatible HP-CPR training and protocols across agencies.
4	Initial HP-CPR Training and Joint Training Event Delivered	Complete initial HP-CPR training for all agencies and conduct at least one multi-agency joint training/simulation event bringing together crews from participating agencies.
5	Regional Scorecard Published	Produce and distribute a first regional performance scorecard comparing (de-identified as needed) agency and regional benchmarks.
6	Sustained Regional Improvement Cycle	Establish a recurring (e.g., quarterly) work-group meeting cadence with tracked action items and demonstrated year-over-year regional survival-rate improvement.

8. Cardiac Arrest Registry to Enhance Survival (CARES) Program

Enrolling in and sustaining participation in a standardized, national out-of-hospital cardiac arrest registry to benchmark performance and drive system-wide improvement.

1. Overview & Purpose

The Cardiac Arrest Registry to Enhance Survival (CARES) is a nationally recognized, standardized OHCA registry that captures dispatch, EMS, and hospital-outcome data using the Utstein reporting framework. Participation allows an agency to benchmark its cardiac-arrest performance against similar agencies nationwide and track survival trends over time using validated, risk-adjusted metrics.

This section outlines the components necessary for any Fire/EMS agency, regardless of size, to successfully enroll in and sustain participation in CARES (or an equivalent state/regional cardiac-arrest registry where CARES is unavailable).

[Resuscitation Academy Toolkit CARES](#)

2. Cost Considerations & Budgeting

- **Registry Participation Fees:** Budget for annual CARES site-participation fees (cost varies by agency call volume and jurisdiction; confirm current fee schedule with the CARES coordinating center) and any state/regional registry membership dues.
- **Data Abstraction Staffing:** Budget for dedicated (or shared/part-time) data-abstraction staff time to enter every qualifying cardiac-arrest case into the registry accurately and within reporting deadlines.
- **Software/Interface Costs:** Include costs for ePCR-to-registry data-interface tools or manual entry-portal licensing, and any IT support needed to maintain the data feed.
- **Hospital Data-Sharing Costs:** Account for any costs associated with establishing hospital outcome-data feeds (survival to discharge, neurological status), which may require IT/legal support to formalize.

3. Equipment & Procurement

- **ePCR Compatibility:** Confirm the agency's ePCR system can export data fields compatible with CARES Utstein variables, or plan for a manual/hybrid entry workflow if not.
- **Data Validation Tools:** Use built-in or supplemental data-validation/quality-check tools to catch missing or inconsistent fields before submission deadlines.

4. Agency Cooperation & Interagency Coordination

- **Hospital Partner Agreements:** Establish data-sharing agreements with all receiving hospitals in the service area to obtain patient-outcome data, which is essential to CARES' value (linking pre-hospital care to outcomes).
- **Regional/State Registry Coordination:** Coordinate with the state EMS office and, where applicable, a regional CARES coordinating site to align reporting timelines and avoid duplicate data requests.
- **Multi-Agency Data Consistency:** Where multiple agencies respond to the same incidents (first responder engine plus transporting ambulance), coordinate to avoid duplicate or conflicting case entries.

5. Administrator & Command Staff Buy-In

- **Leadership Commitment to Data Transparency:** Secure agency leadership agreement to participate in national benchmarking even when early results may show below-benchmark performance — framing this as the necessary first step to improvement, not a liability.

- **Staffing & Budget Approval:** Obtain budget approval for registry fees and abstraction staffing as a recurring, not one-time, operating cost.
- **Integration with Existing QI Structure:** Ensure CARES data feeds directly into the QI/QA processes described in Sections 5 and 6, so registry participation drives real practice change rather than existing as a standalone reporting exercise.

6. Stakeholder & Community Buy-In

- **Medical Director & Clinical Champions:** Engage the EMS medical director and physician/clinical champions to interpret registry benchmarking data and translate it into protocol and training priorities.
- **Governing Body & Public Reporting:** Share (as appropriate, de-identified/aggregate) CARES performance trends with the governing board and community to demonstrate accountability and justify continued investment in PAD, T-CPR, and HPCPR initiatives.
- **Regional Work Group Alignment:** Use CARES data as the common performance currency for the multi-agency HPCPR work groups (Section 7), enabling apples-to-apples regional comparison.

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Registry Enrollment Completed	Complete the CARES (or equivalent) site-enrollment application, data-use agreement, and initial onboarding with the coordinating center.
2	Data Pipeline Established	Configure ePCR export or manual entry workflow and successfully submit a validated test batch of cardiac-arrest cases.
3	Hospital Data-Sharing Agreements Executed	Formalize outcome-data sharing agreements with all major receiving hospitals in the service area.
4	Full Case Capture Achieved	Reach and sustain 100% capture of eligible OHCA cases into the registry on an ongoing basis, meeting reporting deadlines.
5	Benchmark Report Received & Reviewed	Receive the first CARES benchmarking report and formally review findings with the QI committee and agency leadership.
6	Data-Driven Action Plan	Translate registry findings into specific, tracked improvement actions across the PAD, T-CPR, HPCPR, and QI initiatives described in this plan, with progress reviewed annually.

9. Community CPR Programs

Building sustained, community-wide bystander CPR capability through targeted training, public awareness, recognition, and mobilization — grounded in the Resuscitation Academy Community CPR Toolkit.

1. Overview & Purpose

Bystander cardiopulmonary resuscitation (CPR) can more than double a cardiac-arrest patient's chance of surviving and returning to a normal life, yet in many communities fewer than half of out-of-hospital cardiac arrest (OHCA) patients receive bystander CPR before EMS arrival. For every minute that passes without intervention, a victim's chance of survival falls by roughly 10%, the well-known 'slope of death.' Because most cardiac arrests occur in the home, training members of the public is one of the highest-leverage investments an agency can make in its chain of survival.

A Community CPR Program is the public-facing counterpart to the clinical and dispatch programs described elsewhere in this plan (PAD, T-CPR, HPCPR, QI/QA, and CARES). Its purpose is to close the 'bystander-to-bystander' gap: moving people from simply knowing CPR to actually acting in the moment of a real emergency. This section is modeled directly on the Resuscitation Academy's Community CPR Toolkit and is generalized so that any Fire/EMS agency, regardless of size or budget, can build and sustain a community CPR campaign.

[Resuscitation Academy Toolkit Community CPR](#)

2. Cost Considerations & Budgeting

- **Program Coordination Staffing:** Budget for a dedicated or shared-duty community CPR/public-education coordinator to organize outreach, training events, stakeholder relationships, and media engagement — this role does not require a large budget to be effective, but it does require protected time.
- **Self-Learning & Take-Home Kits:** Budget approximately \$30–\$45 per unit for CPR Anytime-style take-home kits (inflatable manikin plus instructional video) that allow trainees to teach friends and family, extending reach at low marginal cost.
- **Low-Cost Training Alternatives:** Where manikins are limited, plan around low-cost or no-cost manikin substitutes validated in the literature (soccer balls, empty/full 2-liter soda bottles, tennis balls) so training is never bottlenecked by equipment shortages.
- **Recognition & Incentive Budget:** Set aside a modest annual line item for low-cost recognition items (lifesaver stickers/pins, patches, challenge coins for citizens/telecommunicators/officers, t-shirts) that meaningfully increase engagement and social sharing at minimal cost.
- **Event & Materials Costs:** Budget for printed materials (flyers, factsheets), venue/booth costs at community events, and translation/localization of materials for non-English-speaking populations.
- **Funding Sources:** Pursue city council/county commission appropriations, EMS/health-department grants (e.g., resources such as [emsgrantshelp.com](#)), civic and service-club grants (Kiwanis, Rotary, Knights of Columbus), local business sponsorships (e.g., a 'Lifesaver Latte' promotion), and health-insurer partnerships funding AED placement or member training.

3. Equipment & Procurement

- **Training Manikins & Household Substitutes:** Maintain a core inventory of CPR training manikins for scheduled classes, supplemented by household-item substitutes (soccer balls, soda bottles, tennis balls) for pop-up, ambush, or high-volume events where manikin supply is limited.
- **Take-Home / Self-Learning Kits:** Stock CPR Anytime-style kits, 'CPR in a Box' loaner kits for schools/businesses/community groups, and links to free self-learning tools (e.g., a Learn-CPR style website or app) to extend training beyond instructor-led sessions.

- **Gamified/Feedback Training Tools:** Where budget allows, consider compression-sensor/gamified training tools (e.g., a music-synced compression-feedback device) that increase engagement, particularly with school-age audiences.
- **Crowdsourcing/Mobilization App Integration:** Evaluate integration with a bystander-alert crowdsourcing application (e.g., a PulsePoint-, GoodSAM-, or HeartRunner-type platform) tied to the PSAP so trained citizens can be alerted to nearby cardiac-arrest events and directed to the nearest public AED.
- **AED Location Mapping:** Maintain (or contribute to) a public, shared AED location map/spreadsheet — even a simple, no-cost tool — to support both citizen response and ongoing device-maintenance tracking, complementing the PAD program in Section 1.

4. Agency Cooperation & Interagency Coordination

- **Key Stakeholder Identification:** Identify and engage local EMS/hospital leaders, government officials, medical directors, health department representatives, schools, faith organizations, businesses, health clubs/recreation centers, neighborhood groups, professional/recreational sports teams, and survivor/family advocates as program partners.
- **Cross-Sector Partnership Agreements:** Formalize arrangements with schools, businesses, universities, parks and recreation departments, and community organizations to host regular training sessions, provide venues, or require CPR training for members/employees/students.
- **Data & Registry Coordination:** Where a cardiac-arrest registry (CARES, Section 8) or local database exists, coordinate with its data manager to geocode cardiac-arrest 'hot spots' and target training resources to the neighborhoods, populations, and facility types with the greatest need.
- **Public Safety & Government Alignment:** Coordinate with law enforcement, fire service, and Department of Transportation/Parks & Recreation partners so citizen CPR/AED training complements the PAD-embedded-in-police initiative (Section 2) and public AED placement (Section 1).

Commented [ML31]: Build upon school-based CPR education and training requirements (mandatory legislation, school district policies, etc.) to establish early CPR/AED acceptance, adoption, and expectations.

5. Administrator & Command Staff Buy-In

- **Leadership Endorsement & Data-Driven Case:** Present administrators with local bystander-CPR-rate data and national context (nationally, only about 46% of OHCA patients receive bystander CPR) to make the case that community training is a core survival strategy, not a discretionary outreach activity.
- **Sustained Budget & Staffing Commitment:** Secure recurring, not one-time, budget and staff time — an effective program can scale to available funding, from a minimal self-learning-kit model to a fully staffed mass-training campaign, but requires ongoing rather than episodic investment.
- **Policy Support for Non-Certification Training:** Obtain administrative approval to count brief, non-certification 'compression-only' instruction (e.g., 2-minute drop-in sessions at community events) as a legitimate program activity, not just formal certification courses, to maximize reach.
- **Integration with Municipal Governance:** Regularly brief city/county governance on cardiac-arrest and CPR-training data to help unlock funding for public activation programs, AED replacement costs, and expanded community outreach.
- Build upon school-based CPR education and training requirements (mandatory legislation, school district policies, etc.) to establish early CPR/AED acceptance, adoption, and expectations.

6. Stakeholder & Community Buy-In

- **Targeted Community Outreach:** Prioritize outreach to cardiac-arrest and overdose 'hot spot' locations, racial and ethnic minority communities (who experience lower survival rates and should be reached through culturally sensitive, appropriately translated programming), schools, religious organizations, neighborhood associations, and youth/sports groups.
- **Overcoming Bystander Barriers:** Address the documented barriers to bystander action — bystander effect, fear of causing harm, fear of legal liability, uncertainty about when CPR is needed, and fear of mouth-to-mouth contact — directly in training messaging (e.g., 'you cannot hurt a victim, you can only

Commented [ML32]: Consider peer-to-peer CPR/AED education and training in underserved communities—especially those for who speak languages other than English.

help'; educate on state Good Samaritan protections; emphasize hands-only CPR as an equally effective, lower-barrier option).

- **Clear, Consistent Messaging:** Adopt simple, tested phrasing across all materials — such as 'Call. Push. Shock.' or the AHA's 'Call 911, then push hard and fast to the beat of Stayin' Alive' — to make the message memorable and easy to teach.
- **Recognition of Citizen Rescuers:** Recognize bystanders, telecommunicators, and responders through lifesaver dinners, press coverage, social media features, public presentations to council/school boards, and AED signage reading 'This AED was used to save a life' — always confirming rescuer/survivor comfort with public attention first.
- **Media & Public Awareness Campaigns:** Use press releases, social media, local news features, and survivor storytelling to build community awareness; align major events with national/international observances (World Restart a Heart Day, National CPR/AED Awareness Week, Sudden Cardiac Arrest Awareness Month) to amplify reach through existing publicity.
 - Utilize AED Registry to broadcast CPR/AED messaging and education to AED owners
- **Under-Leveraged Community Resources:** Engage local celebrities, elected officials, athletic teams and their trainers, universities/community colleges, and civic organizations (Rotary, Kiwanis, Knights of Columbus) as force-multipliers for both training delivery and funding.

Commented [ML33]: Utilize AED Registry to broadcast CPR/AED messaging and education to AED owners

7. Implementation Milestones

A minimum of five milestones is required to move this initiative from concept to sustained operation. Agencies should adapt timeframes to local resources while preserving the sequence below.

#	Milestone	Description / Success Criteria
1	Community Needs & Stakeholder Assessment	Identify local bystander-CPR rates, cardiac-arrest/overdose hot spots, and priority populations; build a stakeholder map of potential community, government, and business partners.
2	Program Design & Messaging Adopted	Select consistent training messaging (e.g., 'Call. Push. Shock.'), training format(s) (mass training, ambush training, self-learning kits), and address common bystander barriers in course materials.
3	Training Infrastructure & Partnerships Established	Secure manikins/substitute training tools, take-home kits, venues, and formal partnerships with schools, businesses, faith organizations, and community groups to host recurring sessions.
4	Initial Training Rollout	Deliver an initial series of targeted and general-public training events (including at least one large-scale or event-based training), tracking number of citizens trained by location and population.
5	Recognition & Media Program Launched	Implement a citizen/telecommunicator recognition process (e.g., lifesaver dinner, press features, social media recognition) and issue at least one coordinated media/public-awareness campaign.
6	Sustained Program & Outcome Tracking	Establish an annual training calendar, ongoing funding source(s), and a process to track bystander-CPR rate trends over time (ideally linked to CARES data in Section 8) to demonstrate program impact and guide continuous improvement.

Absolutely, but we would be remiss to ignore budgetary costs as a potential barrier. This is nothing more than a "consideration" component. As a Chief, cost is ALWAYS a question I have to ask, "what is it going to cost for me to do this" and if I hear "nothing" that is inaccurate because there are tangible costs associated with change